

Standard Operating Procedure

Patient Registration, Identification and Admission

«Hospital Name» — Supports AAC.2.a, AAC.2.b, AAC.2.c

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» registers every assessed patient (including unidentified patients), assigns a unique identification number at the end of registration, and decides admission or acceptance only when the required service can be provided — without ever delaying emergency life-saving treatment for that decision.

2. Scope

Applies to every registration and admission point: front office, OPD registration, emergency, day-care, and in-patient admission desks.

3. Responsibility

Registration / Front Office Staff: register every assessed patient and verify identity.

Treating Doctor: authorises every admission and decides service-acceptance after any needed emergency stabilisation.

Medical Records: maintains the unique-number master index and checks for duplicates.

4. Procedure

4.1 Registering a patient, including unidentified patients

1. Verify identity using government photo ID where available, or two identifiers stated by the patient or accompanying person. This two-identifier check follows The Joint Commission's National Patient Safety Goal on patient identification — see Section 8, Reference 1.
2. For an unidentified patient (e.g. found unconscious, no ID), register using the unidentified-patient process: assign a temporary descriptive identifier (e.g. "Unknown Male, approx. 40 yrs, brought by police") and update to the real identity as soon as it becomes known, cross-referencing the unique number already issued.
3. Obtain general consent for treatment at the point of entry, and explain its scope to the patient or family before proceeding.

4.2 Assigning the unique identification number

1. At the end of the patient's first-ever registration with the hospital, generate one unique identification number. Never assign a second unique number to a patient who already has one — search the master index by name, date of birth, and other identifiers first.
2. For later visits, a visit/encounter number may be created in addition, but it must be linked to the existing unique number, not replace it.

3. Medical Records runs a duplicate-number check periodically and logs the result, per the Duplicate-Number Audit Log.

4.3 Deciding acceptance — and never delaying emergency care for it

1. Check the current service directory to confirm the hospital can provide the required service before accepting a non-emergency patient.
2. For an emergency presentation, begin life-saving treatment immediately — the acceptance decision is made only after the patient is stabilised, never before.
3. If, after stabilisation, the hospital cannot continue care, arrange safe transfer per the hospital's Patient Transfer and Referral SOP.
4. If a non-emergency patient is not accepted, log the presentation, the reason, and the advice or referral given in the Non-Accepted Presentation Log.

5. Records

This SOP is supported by Records 1–9 of the AAC.2 Required Records pack: the Registration & Admission Guidance Log, Sample Registration & Consent Audit, Admission Authorisation Audit, Unique Number Master Index, Cross-Department Unique Number Trace, Duplicate-Number Audit Log, Non-Accepted Presentation Log, Emergency Stabilisation-Before-Acceptance Audit, and Service Directory Availability Log.

6. Related Documents

- HCO_AAC_2_v2_REWRITE_DRAFT — AAC.2 policy
- AAC.1 Required Records — Service Directory (Record 1)
- SOP — Patient Transfer (In, Out) and Referral

7. Review

Reviewed annually, or sooner if the registration system or service directory changes.

8. References

- The Joint Commission. (2026). National Patient Safety Goals: Identify patients correctly (Goal NPSG.01.01.01). <https://www.jointcommission.org/en-us/standards/national-patient-safety-goals>